INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Irinotecan · every three weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00213
Version 7

> Irinotecan on Day 1 every 21 days while beneficial and tolerable. There is no home chemotherapy pump. Blood counts are checked weekly, including between infusion visits.

Treatment stages

01 **A 21-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts deoxyribonucleic acid (the cell’s genetic material) (DNA) copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

Why this option?

Irinotecan alone is used for advanced bowel cancer after a fluorouracil-containing treatment has failed. Your team explains the aim, alternatives and how benefit will be reviewed.

A plan that can change

For this advanced-cancer treatment, discuss cancer control, symptom relief and how benefit will be reviewed. Ask about other options, treatment burdens and your priorities.

Cunningham et al. 1998: irinotecan after fluorouracil failure

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

In a historical trial after fluorouracil had stopped working, irinotecan helped people live longer and improved several cancer-related symptoms compared with supportive care alone. Treatment also caused side effects. The study does not compare irinotecan with every option used today.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, recovery and review

Irinotecan · every three weeks

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 21-day reference cycle

D1 D21

Irinotecan on Day 1 every 21 days while beneficial and tolerable. There is no home chemotherapy pump. Blood counts are checked weekly, including between infusion visits.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00213 v7 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Irinotecan · every three weeks: treatment and checks

Advanced / metastatic colorectal cancer

Irinotecan on Day 1 every 21 days while beneficial and tolerable. There is no home chemotherapy pump. Blood counts are checked weekly, including between infusion visits.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00213	Baseline blood count, kidney and liver profile	Review results before treatment	Weekly blood count; kidney and liver tests before each cycle	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Bowel symptoms and hydration NCCP 00213	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day; act using your written plan	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Cancer response and treatment choices NCCP 00213	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1 **A 21-day reference cycle**

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Visits, access and care between treatments

STEP 1

Paper copies, wallet QR and contacts

The example telephone numbers on the wallet card are placeholders. Your team must replace them with working daytime and out-of-hours numbers before patient use. Keep the card with you; the QR opens the regimen information, not your confidential medical record.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements.

Preparing access

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

The exact delivery plan

Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 21-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Radiotherapy relationship

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward chemotherapy	 **Day 1** intravenous (IV)
Early cramps / sweating	 **Day 1** Tell the nurse
Delayed diarrhoea	 **Days 2–21** Can occur after 24 hours
Blood counts / infection risk	 **Days 1–21** Blood tests and symptoms guide review
Energy and bowel symptoms	 **Days 1–21** Variable timing

Your planned visits: Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 21-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Care continues: Continue the bowel-symptom plan, blood tests and follow-up arranged by your team; report delayed diarrhoea promptly. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

> **Why these bands stay broad:** NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00213 v7 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Know the contact number and the next planned visit.

Day 2–10: Care between visits

Follow the skin-care or bowel-symptom plan provided. There is no pump to remove. Call early for symptoms; do not wait for the next appointment.

Day 11–21: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

Support around the medicines

1 Before treatment

The team checks blood counts, kidney/liver function, symptoms, allergies and medicines. Weekly blood counts are part of this irinotecan plan.

2 Bowel symptoms and fluids

Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

3 Comfort and daily life

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

4 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Weekly blood counts remain part of the irinotecan plan between visits.

Symptoms in context

Report what changed and how it affects you.

Living with treatment

Use the support available.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Blood tests between infusion visits

The infusion and test schedules are different.

Weekly blood counts

Irinotecan is given every 21 days, but the full blood count is required weekly, including the weeks without an infusion. Ask where the blood is taken, who checks the result and what to do if an appointment is missed.

Before the next cycle

Kidney and liver tests are required before each cycle. The team also reviews bowel symptoms, hydration, medicines and recovery before confirming more treatment.

Early versus delayed diarrhoea

Tell the nurse immediately about cramps, sweating or diarrhoea during or soon after infusion. Delayed diarrhoea starting after 24 hours uses a separate written rescue plan. Contact urgently if it persists, or if fever, severe pain, dehydration or reduced urine occurs.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

Will I have a pump at home?

There is no home fluorouracil pump in this regimen. Your team chooses a cannula or central line based on your veins and wider treatment plan.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

Try explaining the plan back: Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · irinotecan

Active medicines: Irinotecan

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row.

Medicine	Dose / route	When
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	Per prescribed irinotecan protocol	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.
- The team checks blood counts, kidney/liver function, symptoms, allergies and medicines. Weekly blood counts are part of this irinotecan plan.
- Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.
- Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.
- Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.
- Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Weekly blood counts remain part of the irinotecan plan between visits.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair / scalp

Irinotecan

Hair thinning or loss may occur. Ask about practical support.

2 Mouth / swallowing

Irinotecan

Report ulcers or pain affecting eating or drinking.

3 Breathing / reaction

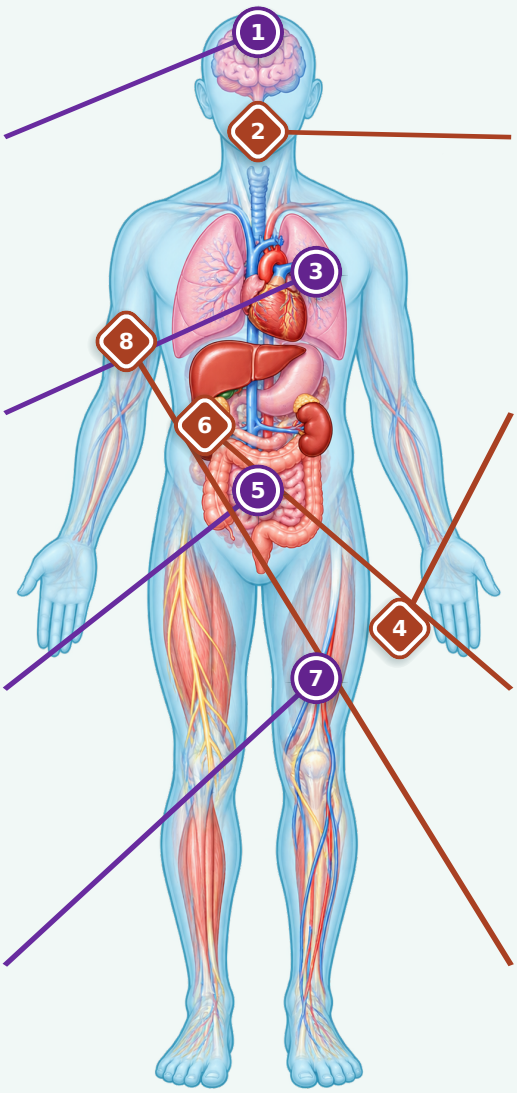
Irinotecan

New breathlessness, wheeze or severe breathing difficulty needs urgent assessment.

4 Sweating / early reaction

Irinotecan

Cramps, sweating or diarrhoea during infusion: tell the nurse immediately.



5 Bowel / stoma

Irinotecan

Delayed diarrhoea after 24 hours needs your written plan; fever or dehydration needs urgent advice.

6 Hydration / urine

Irinotecan-related fluid loss

Reduced urine, dizziness or inability to replace fluids needs prompt advice.

7 Blood / infection

Irinotecan

Keep weekly blood tests. Fever, shivers or sudden illness: call oncology immediately.

8 Infusion access

Treatment delivery

Report pain, swelling or leakage during infusion. This regimen has no home pump.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00213 v7 · official regimen**
   https://healthservice.hse.ie/documents/6553/00213_Irinotecan_Monotherapy.pdf
2. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
3. **Cunningham 1998 primary trial**
   https://pubmed.ncbi.nlm.nih.gov/9807987/
4. **NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq